

Proteinuria and leukopenia have been reported rarely. Administration of the drug should be discontinued if granulocytopenia occurs. When rare, serious with Griseofulvin, they are usually associated with high dosages, long periods of therapy or both.

Symptoms and Treatment of Overdose:

Treatment is unlikely to be required in acute cases. Treatment of over dosage should be symptomatic.

Storage Condition:

Keep container tightly closed. Store in a dry place (below 30°C). Protect from light.

Pack Size:

Plastic container containing 30 tablets.
Blister pack of 10x10's and 100x10's.

Product Registration Number:

MAL19860837AZ

The logo for ROYCE, featuring the word "ROYCE" in a bold, serif font with a registered trademark symbol (®) to its upper right.**Product Holder / Manufactured by:**

ROYCE PHARMA MFG SDN BHD. 650435-X
PT 1663, Nilai Industrial Estate,
71800 Nilai, Negeri Sembilan. Malaysia.

Revision date: 200917

ROYCE®

GRISEOFULVIN TABLET 125MG
(Griseofulvin-Antifungal)**Presentation:**

A white colour, round, flat face, bevel edge tablet, plain on one side and a break line on the other.

Each tablet contains: Griseofulvin (Micronized) 125mg.

Pharmacological Information:

Griseofulvin inhibits fungal cell mitosis by causing disruption of the mitotic spindle structure, thereby arresting the metaphase of cell division. It is deposited in the keratin precursor cell or skin, hair and nails, rendering the keratin resistant to fungal invasion. As the keratin is shed, it is replaced with healthy tissue.

Griseofulvin is irregularly absorbed over a prolonged period from the gastro intestinal tract. It is deposited in keratin precursor cells and is concentrated in the stratum corneum of the skin and in the nails and hair, thus preventing fungous invasion of newly formed cells. Griseofulvin which is excreted in the urine. A large amount of a dose of griseofulvin is excreted in the sweat.

Serum concentrations of griseofulvin have been reported to range from 0.6 to 1.8 ug per ml 4 hours after doses of 0.25 to 1 g. The absorption of griseofulvin is greatly increased by reducing its particle size.

Indication:

The treatment of fungal infection of the skin, scalp, hair or nails where topical therapy is considered inappropriate or has failed.

When Griseofulvin is given orally for systemic treatment of ringworm infections, it enables newly formed keratin of the skin, hair and nails to resist attack by the fungi. As the new keratin extends, the old infected keratin is shed. Griseofulvin is not effective against the dermatophytes causing ringworm (tinea), including *microsporum canis*, **Trichophyton rubrum** and **T. verrucosum**. Griseofulvin is not effective in infections caused by *Candida albicans* (monilia), *Aspergilli*, *Malassezia furfur* (Pityriases versicolor) and *Nocardia* species.

Dosage and Administration:

Griseofulvin should be taken after meals for optimum absorption.

Adults: 500-1000mg daily, but not less than 10mg/kg bodyweight, taken as a single dose or in divided doses for patients who respond poorly.

Children: 10mg/kg bodyweight daily in divided doses. For hair or skin infections at least 4 weeks treatment is required but for nails treatment should be extended up to 6-12 months. Treatment should in general be continued for at least 2 weeks after signs of the infection have disappeared.

Contraindication:

This drug is contraindicated in patients with porphyria, hepatocellular failure and in individuals with a history of hypersensitivity to Griseofulvin. Griseofulvin may cause liver disease to deteriorate and liver function should be monitored in such conditions.

Warning and Precautions:

Patients on prolonged therapy with any potent medication be under close observation. Periodic monitoring of organ system function, including renal, hepatic and hematopoietic should be done. Since Griseofulvin is derived from species of *Penicillium*, the possibility of cross-sensitivity with penicillin exists, however, known penicillin-sensitive patients have been treated without difficulty.

Since a photosensitivity reaction is occasionally associated with Griseofulvin therapy, patients should be warned to avoid exposure to intense natural or artificial sunlight. Lupus erythematosus or lupus-like syndromes have been reported in patients receiving Griseofulvin. Griseofulvin decreases the activity type anticoagulants. So patients receiving these drugs concomitantly may require dosage adjustment of the anticoagulant during and after griseofulvin therapy. Barbiturates usually depress griseofulvin, producing such effects as tachycardia and flush.

Prophylactic usage:

Safety and efficacy of Griseofulvin for prophylaxis of fungal infections have not been established.

Use in pregnancy:

The safety of this drug during pregnancy has not been established.

Animal Reproduction Studies:

It has been reported on literature that Griseofulvin was found to be embryotoxic and teratogenic on oral administration to pregnancy rats. Pups with abnormalities have been reported in the litters of a few bitches treated with Griseofulvin.

Suppression of spermatogenesis have been reported to occur in rats but investigation in man failed to confirm this.

Side Effects:

When adverse reactions occur, they are most commonly of hypersensitivity type, such as skin rashes, urticaria and rarely, angioneurotic edema and may necessitate withdrawal of therapy and appropriate counter measures. Paresthesias of the hands and feet have been reported rarely after extended therapy. Other side effects reported occasionally are oral thrush, nausea, vomiting, epigastric distress, diarrhoea, headache, fatigue, dizziness, insomnia, mental confusion and impairment of performance of routine activities.